Kreative Technologies, LLC (Kreative) congratulates the Department of Health and Human Services (HHS) Centers for Medicare & Medicaid Services (CMS) for releasing the Health Technology Ecosystem Request for Information (RFI). By engaging a broad spectrum of stakeholders—including patients, caregivers, providers, payers, technology vendors, data providers, networks, and value-based care organizations—CMS can gain critical insights from diverse and valuable perspectives. This collaborative approach stands to provide both CMS and industry with a comprehensive and current view of the evolving health technology landscape. The resulting knowledge will support progress in modernizing technology infrastructure, expanding beneficiary access to impactful digital tools, improving data availability across stakeholders, and ultimately advancing the health outcomes for numerable patients.

Kreative is an IT Services company founded in 2013 on a desire to offer a collaborative, innovative, and creative source for software development, data solutions, cloud and systems integration, cybersecurity, and Artificial Intelligence (AI) and advanced analytics solutions. With a strong footprint in Military Health Systems and hard-won expertise in health IT modernization, electronic health records (EHR), cybersecurity, and systems integration, Kreative is uniquely positioned to support CMS in this information gathering initiative aimed at improving healthcare access, quality, and interoperability. Our work places us at the forefront of digital transformation in federal health care—where complex environments, sensitive data, and diverse patient populations demand solutions that are secure, scalable, and user-centric. Kreative is committed to empowering agencies like CMS through secure, compliant, and modern IT solutions that improve outcomes for providers, payers, and patients alike.

We believe the transition to technology-enabled care must be anchored in strong data standards, human-centered design, and infrastructure that reaches the most underserved. Many of the lessons learned within the military health space can inform CMS's strategy—especially around secure access, patient empowerment, and real-time decision support. We believe that digital innovation in federal health care must always be grounded in equity, usability, and trust. We are proud of our contributions to the military healthcare environment and stand ready to support CMS in bringing similar transformation to Medicare and Medicaid beneficiaries nationwide.

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In response to the Health Technology Ecosystem RFI, Kreative puts forth the following inputs for **Section E. Technology Vendors, Data Providers, and Networks**.

> ***TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers****?*
>
> The technology challenges in healthcare are complex and multifaceted. No single product can address all the obstacles, issues, and systemic problems that exist across the sector. Effective solutions must be built on highly interoperable, user-centered software that prioritizes real-world usability and inclusivity.
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> Strategic incentive programs—both short- and long-term—can help shape the market in two key ways. First, by encouraging the development of high-quality software that addresses the most pressing problems in healthcare, not solely those that yield the highest profits. These programs should reward innovation that improves access and care quality

for underserved populations. In the near term, expanding public access to digital health portals can immediately increase both market size and healthcare accessibility.

To drive interoperability, one approach is to offer short-term incentives for collaborative data-sharing and integration projects. These efforts could be supported by longer-term benefits, such as tax incentives, that reward organizations for sustaining and expanding interoperability over time. Additionally, establishing clear standards for security and interoperability would not only support system-wide consistency but also enhance the credibility and appeal of compliant solutions—improving public trust, increasing adoption, and ultimately driving better health outcomes.

TD-2. Regarding CMS Data, to stimulate developer interest—

a. What additional data would be most valuable if made available through CMS APIs?

Anonymized health datasets for disease patterns would be a massive benefit to technology companies looking to build improved health pattern datasets. This would empower better digital therapeutics, diagnostics, and monitoring programs.

TD-3. Regarding digital identity implementation:

a. What are the challenges and benefits?
b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?
c. What impact would mandatory use of the OpenID Connect identity protocol have?

On the benefits side, adopting strong, standardized digital identity credentials (such as those aligned with NIST 800-63-3 IAL2/AAL2 standards) enhances trust, security, and interoperability across systems. This enables CMS to better safeguard sensitive health and beneficiary data, streamline access to services, and improve user experience through single sign-on (SSO) and reduced identity fraud. Challenges include ensuring equitable access for all populations, addressing technical integration complexities across legacy systems, managing vendor lock-in risks, and maintaining compliance with federal privacy and accessibility requirements.

Requiring digital identity credentials from approved Credential Service Providers (CSPs)—such as Login.gov, ID.me, or CLEAR—would strengthen CMS's cybersecurity posture by reducing unauthorized access and ensuring higher assurance of user identities during data exchange. It would also enable more secure interoperability with external partners and systems by providing consistent, verifiable identity assertions.

Mandating the use of the OpenID Connect (OIDC) protocol would further benefit CMS by standardizing authentication and enabling scalable, secure federation of identities across healthcare and government entities. OIDC's support for modern security features and compatibility with OAuth 2.0 would also improve integration with cloud-based services and APIs. However, the requirement could impose initial technical and policy alignment

burdens on partners unfamiliar with the protocol, requiring targeted support and transition planning.

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

CMS can better encourage the use of open, standards-based, publicly available APIs by aligning incentive programs to reward interoperability and long-term collaboration. This includes offering financial or regulatory benefits to organizations that adopt and maintain open APIs, particularly those that support underserved populations and improve access to care. Additionally, CMS can establish certification standards that elevate the visibility and credibility of open API solutions, encouraging industry adoption. By coupling short-term incentives with sustained benefits—such as tax credits or preferred contracting status—CMS can shift market behavior away from proprietary systems and toward a more open, accessible health technology ecosystem

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

The government could host the directory and charge a minimal licensing fee for companies seeking to utilize the information, while rigorously enforcing a standard of what it may be used for (like an academic board's IRB, which determines if the ethics of a study are valid). This can be an elected or appointed body, or any arbitrating body or standard set forth by the government. If the users (patients and caregivers) must bear a significant cost burden, it will dissuade users who may benefit the most from the applications.

TD-9. Regarding certification of health IT:

a. What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?

b. What would be the drawbacks?

APIs are essential to building the CMS technology ecosystem. They enable interoperability across companies and infrastructure, reduce manual work, empower patients by giving them access to functionality and data, and allow innovation through third-party developers. In many ways APIs will become the foundation for connected healthcare.

CMS should balance its emphasis on APIs with foundational software. Modern platforms build for clinical workflow and patient support (medical and administrative) enable the core functionality behind how healthcare is planned and delivered. If there is inconsistent API design and delivery, or if core systems are neglected, it could lead to poor user experiences and inefficient healthcare operations and processes.

TD-12. Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

CMS should endorse non-CMS data sources that meet a certain rigorous standard of quality, which could be determined by industry or government. This would require the government to evaluate datasets but would increase the collaboration within industry and empower digital health insights.

Specific non-CMS data sources that should be assessed for integration and collaboration include commercial Healthcare Information Exchanges, payer databases, social services platforms, and community-based networks. As the CMS assesses these various stakeholders and datasets, it needs to consider data quality, data standards, interoperability, privacy, and accessibility.

TD-18. Information blocking:

b. What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?

CMS should consider the following innovations in the area of information blocking:

- Expand FHIR API adoption to enable seamless data exchange between systems and patient apps.
- Incentivize real-time data sharing, like hospital admission/discharge alerts and EHR-to-EHR notifications.
- Link compliance to reimbursement by tying information blocking enforcement to participation in CMS programs.
- Promote patient access tools like Blue Button 2.0 and universal portals for Medicare/Medicaid beneficiaries.
- Support participation in national data networks such as TEFCA to improve interoperability.
- Fund AI tools for data normalization to standardize and clean health data for sharing.
- Standardize SDOH data sharing to integrate social and clinical care for chronic disease prevention.

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In conclusion, Kreative offers the following recommendations for CMS to consider as it continues to shape policy and implement technological frameworks:

- **Mandate Standards-Based Interoperability**
  Promote adoption of FHIR-based APIs and enforce data sharing protocols that mirror those required across DHA systems to ensure seamless integration of devices and platforms. Consider interoperability incentives that encourage private technology vendors to develop open source/shared interoperable segments/functions. Use these incentives to enforce a

rigorous standard among businesses – improving operational efficiency, security, and perceived credibility across the entire ecosystem

- **Prioritize Security and Zero Trust Architecture**
  Integrate cybersecurity best practices drawn from DHA's zero-trust architecture and RMF (Risk Management Framework) models when approving digital tools for Medicare use.
- **Expand Value-Based Reimbursement for Digital Care**
  Enable CPT/HCPCS code expansion for remote patient monitoring and digital therapeutics, with reimbursement models that reward proven outcomes and sustained patient engagement.
- **Design care and supporting infrastructure around wellness, and the prevention and management of chronic disease.**
  Leverage AI and analytics to provide preventative care to at risk populations, engage citizens through mobile apps and patient portals, provide telehealth for medical care and mental wellness, design digital therapeutics and clinical decision support tools around electronic health records.
- **Make the Patient the Data Owner**
  Emulate patient portal features that allow patients to access and manage their health data directly—enhancing transparency and engagement. Utilize educational resources presented in consumable packaging to further empower the users to understand and act on their health data. In the long term, this empowers the individual to manage and improve their own health.
- **Bridge the Digital Divide with Targeted Investments**
  Fund broadband expansion, device access, and digital literacy programs for rural and underserved populations to ensure no beneficiary is left behind.
- **Establish a Digital Health Evaluation Framework**
  Support a CMS-led evaluation model for usability, clinical impact, and data integrity before new digital tools are added to the Medicare ecosystem.
- **Launch CMS-DHA Innovation Pilots**
  Consider CMS/DHA joint pilot programs that bring together Medicare and TRICARE learnings to advance remote care for chronic disease, aging in place, and care transitions.